TCF induction followed by carboplatin and radiotherapy

This chemotherapy is given before definitive treatment to shrink or control the head and neck cancer. The team explains how induction fits your overall treatment aim and why this particular schedule is appropriate.

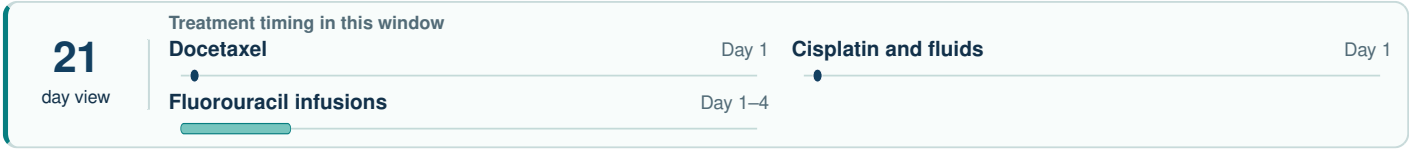
Head and neck cancer: induction chemotherapy · NCCP 00323 v7b · Document revised 10 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

This guide covers NCCP 00323 induction and explains the next treatment stage. Doses and dates are confirmed by your team. Later treatment needs its own agreed prescription; do not carry induction medicines or support instructions into it automatically.

My overall treatment

Every 21 days for 3 planned induction cycles.



Induction
Day 1: docetaxel and cisplatin. 5-FU on Days 1–4; 96-hour infusion or the prescribed daily 22-hour alternative.

Next treatment
Later carboplatin weekly with radiotherapy for seven weeks; the team confirms the start date.

A typical 21-day cycle

Day 1	Day 2–4	Day 5–21
three-drug treatment begins	Fluorouracil continues	Recovery and review

Timing may change after review. Side effects can occur on any day; there are no guaranteed safe days.

My tests and checks

When	What is checked	Timing / reason
Before	**Bloods and preparation** NCCP / individual plan	Blood count, kidney/liver tests and dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) assessment; electrocardiogram (ECG) and hearing tests when indicated. Confirm steroid and pump instructions.
During	**Tests before each cycle** NCCP	Blood count and kidney/liver tests; review hearing, nerves, mouth, bowels and hydration.
After / response	**Recovery and next phase** NCCP / individual plan	Response and recovery review; scans and further treatment follow your individual plan.

Between treatments

- Confirm whether your prescription uses one continuous 96-hour fluorouracil infusion or four daily 22-hour infusions; the unit writes down connection, change and disconnection times.
- Report fever or sudden illness promptly. Follow the written support plan and do not wait for the next visit if symptoms worsen.

Bring your medicine list, appointment calendar and verified urgent contacts. Check steroid and growth-factor instructions.

When to get help
Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.
Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321
EXAMPLE ONLY — team must replace both numbers before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Hair and scalp
Docetaxel
Hair thinning or loss can occur. Ask about scalp care and practical support.

2 Ears and hearing
Cisplatin
Ringing or hearing loss can persist (ototoxicity). Report a change before the next dose.

3 Mouth and throat
Docetaxel + 5-FU
Soreness or ulcers (mucositis) can make eating and drinking difficult. Contact the team for pain or poor intake.

4 Heart and chest
5-FU
Chest pain can signal a heart effect (cardiotoxicity). Seek urgent assessment during or after the infusion.

5 Blood and infection
Docetaxel + cisplatin + 5-FU
Low white cells can cause serious infection; low platelets cause bruising or bleeding; anaemia causes fatigue. Call for fever or sudden illness.

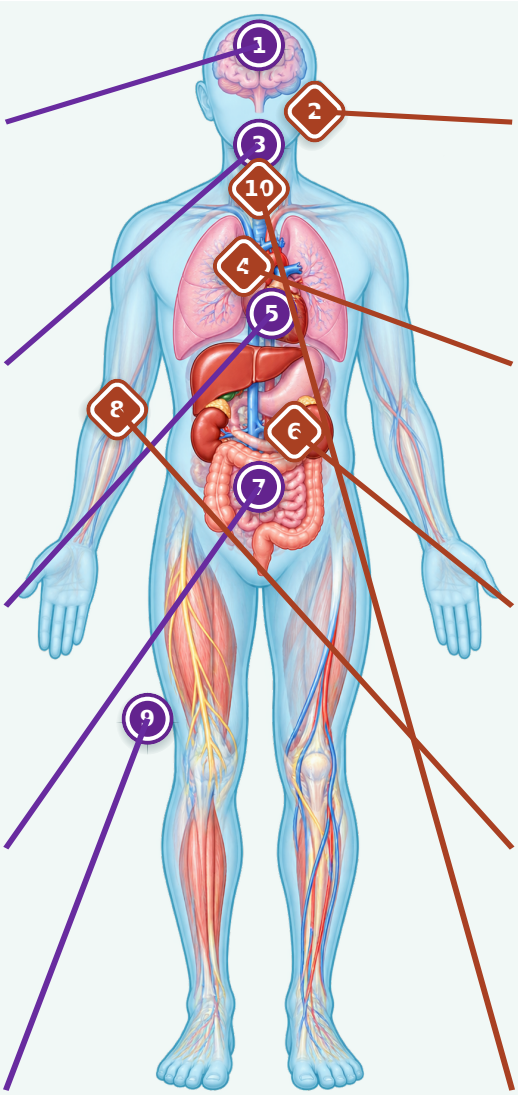
6 Kidneys and fluids
Cisplatin
Kidney injury and mineral changes can occur. Prescribed fluids and blood tests help protect you; report reduced urine or inability to drink.

7 Stomach and bowel
All three chemotherapy drugs
Nausea and vomiting can occur; 5-FU may cause severe diarrhoea. Persistent symptoms can cause dehydration and need prompt help.

8 Skin, nails, hands and feet
Docetaxel + 5-FU
Docetaxel may affect nails and retain fluid; 5-FU may cause painful palms or soles (hand–foot syndrome). Report swelling, pain or skin breakdown.

9 Hands, feet and nerves
Docetaxel + cisplatin
Numbness or tingling (peripheral neuropathy) can persist. Report difficulty with buttons, walking or balance before another dose.

10 Later radiation area
Radiotherapy: later phase
Dry mouth, taste change, sore skin and swallowing difficulty can build up. Some dental, saliva and swallowing effects last; follow the radiation care plan.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call oncology immediately for fever, shivering, sudden illness, unusual bleeding, persistent vomiting, severe diarrhoea, severe mouth pain or inability to drink. Chest pain during or after fluorouracil, sudden confusion, or unexpectedly early severe symptoms needs urgent assessment even if DPD testing was normal. Report reduced urine, hearing change, ringing in the ears or new numbness promptly. Severe chest pain, severe breathing difficulty, collapse or a seizure is an emergency: call 112 or 999.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

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Keep your treatment details to hand

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Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

TCF induction

Every 21 days × 3 induction cycles

Phase: _____ Planned cycles: 3

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

BACK / TREATMENT RECORD

My cycle record

Team stamps after each completed cycle.

C1	C2	C3
Date / initials	Date / initials	Date / initials

My treatment information

Plan changed? Ask the team to update this card.

CUT outer border

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____

Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.